



INFLUENCE OF HEALTH SUPERSTITIONS ON THE HEALTH- SEEKING BEHAVIORS OF ATENEO DE ZAMBOANGA UNIVERSITY SENIOR HIGH SCHOOL STUDENTS

In partial fulfillment of our requirements in
STM 130 (Research Project 2),



Submitted to
MS. NOVA JOY R. SALAJUG, LPT

Research Instructor
of the Research and Immersion Department

Submitted by:
ABDUA, AISHA RAFIAH I.
HAMISANI, KIMBERLY H.
JUMLAIL, YUZRA H.
MAHILUM, ABEGAIL GEORGIA H.

12 STEM ACQUAVIVA

MARCH 14, 2025

ABSTRACT

Health superstitions are beliefs or practices that are not scientifically supported but still influence individuals' perception and response to health issues. These superstitions shape people's health-seeking behaviors, often leading them to avoid or delay professional medical care for traditional or alternative practices. This research examines the correlation between health superstitions and health-seeking behaviors of senior high school students at Ateneo de Zamboanga University. Using a quantitative research design, data were collected through a survey administered to 118 senior high school students from Ateneo de Zamboanga University. By percentage computation, results show that most senior high school students do not believe in health superstitions and only seek medical care when they are unwell. Spearman rho data analysis reveals a significant weak negative correlation between senior high school students' health superstition level and the resources they turn to for health advice and a significant moderate positive correlation between senior high school students' health superstition level and their inclination to perform actions that are not known to be scientifically proven. These findings suggest that while health superstitions do not have a significant influence on the majority of senior high school students' health-seeking behaviors, the higher the superstition level of the students, the more they turn to their families and friends and the Internet for health information and the higher chance they perform actions that are not backed by science. Further research can explore more common Filipino superstitions and their influence on health-seeking behaviors or the underlying factors that constitute the students' health superstitions.

Keywords: health superstitions, health-seeking behaviors, senior high school students, quantitative research, percentage computation, spearman rho data analysis, correlation

TABLE OF CONTENTS

CONTENT	PAGE NO.
COVER PAGE	1
ABSTRACT	2
TABLE OF CONTENTS	4
LIST OF TABLES	6
LIST OF FIGURES	7
CHAPTER ONE: INTRODUCTION	8
Background of the Study	8
Review of Related Literature	11
<i>Perceptions and Experiences on</i>	11
<i>health</i>	
<i>Attitudes toward Complementary</i>	13
<i>and Alternative Medicine</i>	
<i>Health-Seeking Behaviors</i>	14
<i>Health Literacy</i>	15
Statement of the Problem	16
Conceptual Framework	17
Significance of the Study	17
Scope and Limitations	19
Definition of Terms	20
CHAPTER TWO: METHODOLOGY	23
Research Design	23

Research Locale	24
Research Participants/Subject	25
Sampling Method/Subject Collection	25
Data Gathering Procedure and Instruments	27
Data Analysis	28
Ethical Considerations	28
CHAPTER THREE: RESULTS AND DISCUSSION	30
Demographic	30
What is the health superstition level of the students?	30
What are the health-seeking behaviors of the students?	32
How do health superstitions influence the health-seeking behaviors of the students?	37
CHAPTER FOUR: CONCLUSION AND RECOMMENDATION	41
Conclusion	41
Recommendation	42
APPENDICES	44
Appendix A. Instrument	44
Appendix B. Informed Consent	47

Appendix C. Validation Letters with	49
Comments and Signature	
REFERENCES	51

LIST OF TABLES

TABLE NO.	TABLE NAME	PAGE NO.
Table 1	Health superstitions	30
Table 2	Health-seeking behaviors	32
Table 3	Spearman rho correlation between health superstitions and health-seeking behaviors	37

LIST OF FIGURES

FIGURE NO.	FIGURE NAME	PAGE NO.
Figure 1.1.	Conceptual Framework	17

CHAPTER ONE

INTRODUCTION

This chapter explores the background of the study, including its objectives and key insights. It also presents a review of related literature, which evaluates and synthesizes previous studies relevant to the research. Additionally, this chapter outlines the statement of the problem, detailing the research question that the study aims to address. The significance of the study highlights its potential beneficiaries. The conceptual framework illustrates the relationship between the two variables: health superstitions and health-seeking behaviors. Lastly, the definition of terms clarifies the meanings and contexts of key terms used in the study.

BACKGROUND OF THE STUDY

Health is the feeling of soundness, wellness, and rigorousness. Moreover, it is the ability to do things most people can do (Mirowsky, 2017). According to the World Health Organization (WHO), health is one of the fundamental rights of every living being regardless of race, religion, political belief, economic or social condition. Hence, when people deem themselves ill, they seek an appropriate remedy to treat their bodies and regain their health.

Health-seeking behavior is an action taken by anyone who perceives a need for help in terms of health and attempts to solve their problem (Widayanti et al., 2020). Furthermore, preceding health-seeking behavior is a decision-making process further influenced by factors surrounding the individual making the decision (Oberoi et al., 2016). Ascertained by several research investigations, health-seeking behaviors are affected by various factors not limited to attitudes, intentions, and beliefs (Oberoi et al.,

2016), whereby superstitions fall under.

Superstitions are beliefs about particular events that can not be deciphered by scientific reasoning (Taher et al., 2020). Following this, superstitions may employ religious, cultural, and personal beliefs as basis (Uwayezu et al., 2022). The World Health Organization (WHO) explains that traditional medicine established itself on theories, beliefs, and experiences across diverse cultures with aims to maintain health and prevent and treat illnesses. Consequently, the World Health Organization (WHO) reports that 170 countries have utilized traditional and complementary medicine (T&CM) for centuries as it has been a vital aid for health in households and communities. Across different countries, several studies tackled the influence of superstitions on certain health-seeking behaviors.

In a study by Tung et al. (2023), they investigated superstitions based on cultural beliefs that affect the health-seeking behavior of the Taiwanese population for cataract surgery, specifically the superstitions tied to the seventh lunar month or the “ghost month” in Taiwanese culture. Beliefs about spirits roaming freely among the living resulted in individuals refraining from making significant decisions and risky activities, including elective medical procedures and surgeries, to avoid encountering the said spirits or inviting misfortune. Analyzing data from the Longitudinal Health Insurance Database 2000 (LHID2000), Tung et al. (2023) studied cataract surgery patients from 2001 to 2010, categorizing them by gender and living area and comparing surgeries across Chinese lunar months. The findings revealed a marked decrease in surgeries during the seventh lunar month, especially in urban and rural areas, indicating a strong link between cultural beliefs surrounding the ghost month and health-seeking behavior.

Similarly, Lu et al. (2019) investigated the influence of superstitious beliefs on the influenza vaccine using a cross-sectional survey of Singaporeans conducted via computer-assisted telephone interviews. The findings indicated that 60% of the participants had never received the influenza vaccine, with superstitious beliefs associated with its higher barriers and lower perceived benefits. These factors, in turn, were linked to a decreased likelihood of ever receiving the influenza vaccine. Lu et al. (2019) concluded that superstitious beliefs shaped perceptions of susceptibility and severity of influenza and their attitudes towards vaccination by influencing their health beliefs.

Furthermore, in the Philippines, some Filipinos prefer to follow superstitions from their ancestors. In 2023, Lao discovered that Filipino millennials still acknowledge superstitions based on their traditions as a sign of respect toward their elders despite modern technology debunking. Some Filipinos have deep-rooted attachments to such beliefs with hopes of overcoming circumstances in their lives (*Asian Journal of Behavioural Sciences*, n.d.), which includes their health problems, as some prefer taking the traditional and superstitious route of healing themselves rather than seeking professional help.

Multiple research studies have investigated the influence of superstitions on the health-seeking behaviors of individuals. However, limited research investigated the effects of believing in health superstitions on health-seeking behaviors, specifically on the population of Ateneo de Zamboanga University Senior High School. Hence, this gap led the researchers to the idea of determining the influences of health superstitions on health-seeking behaviors among Ateneo de Zamboanga University Senior High School

students. As the scarcity of studies in this domain raises questions about the unique dynamics that might exist among Ateneo de Zamboanga Senior High School students, comprehending how these students navigate health decisions influenced by the health superstitions they believe in could offer valuable insights into their attitudes and behaviors regarding medical care and explore the intricacy that affects how these students believe, interpret, and act upon health-related superstitions.

Thus, this study aims to address a significant gap in the existing research by delving into the demographic of Ateneo de Zamboanga Senior High School University students with the primary objective of uncovering and interpreting the influence of health superstitions on the health-seeking behaviors of the students. Consequently, this study intends to provide a comprehensive insight into the decision-making processes influenced by health superstitions behind the health-seeking behavior of Ateneo de Zamboanga University Senior High School students. To understand the influence of traditional health superstitions on modern health-seeking behaviors within Ateneo de Zamboanga University Senior High School, the researchers will utilize a comprehensive strategy that incorporates several factors, including social, cultural, and psychological variables.

REVIEW OF RELATED LITERATURE

Perceptions and Experiences on Health

Hussain et al. (2024) conducted a qualitative research study regarding the understanding of dementia within the Bangladeshi community in the United Kingdom. To conduct this, the researchers employed semi-structured interviews in three phases

with twenty-five participants. They investigated how cultural myths, superstitions, and stigma influenced the perception of dementia among participants. The study revealed that participants with dementia and their caregivers held misconceptions about the condition due to their different cultural and religious backgrounds. Moreover, the research showed that these participants preferred seeking assistance from spiritual healers over medical professionals. One limitation of this study may be the homogeneity of the sample, as it focused exclusively on the Bangladeshi community, potentially limiting the generalizability of the findings to other cultural groups.

On the other hand, a qualitative and interpretative study conducted by Milton et al. (2021) talks about the perceptions and experiences of Nigerian mothers on stillbirths. However, this study is limited only to those mothers who have given birth in the Murtala Muhammad Specialist Hospital (MMSH). The study's objective is to explore the experiences and perceptions of stillbirth among mothers from a tertiary medical centre in Kano, Northern Nigeria. The data gathering method that the study used is a semi-structured focus group discussion of open-ended questions about stillbirth, beliefs, experiences, and influences that were held in Murtala Muhammad Specialist Hospital (MMSH), which was conducted for only a day. The study findings emphasize that the Murtala Muhammad Specialist Hospital (MMSH) is a resource-poor tertiary facility serving an ever-growing population increasing strain on the hospital and health workers. Logistical access to resources and clinical care is one of the reasons why MMSH is seen as a resource-poor tertiary facility as it does not include the travel distance, availability of a suitable mode of transport or whether someone was available to take the mother to the facility. There are also less frequently discussed barriers to accessing healthcare such

as societal, and familial perceptions and beliefs to accessing healthcare. An understanding of the importance of intrapartum care in reducing stillbirths is vital due to the high proportion of fetal death occurring during an intrapartum stage. Many patients have said that it is not uncommon for the hospital's labour ward to be full, with reports of mothers being turned away and giving birth on the hospital floors or in car parks. The study identified that mothers have knowledge about self-care during pregnancy, yet certain societal factors prevented that from being their priority.

Attitudes toward Complementary and Alternative Medicine

The study conducted by Yilmaz, F. A., & Uzuncakmak, T. (2023) discussed pregnant women's attitudes toward complementary and alternative medicine and the effect of their superstitions on their psychosocial health levels. The objective of the study is to determine pregnant women's attitudes towards complementary and alternative medicine and how it affects their superstitions on their psychosocial health levels, however, it is limited to pregnant Muslim women. The research design that they used is quantitative research. This study was conducted with 253 pregnant women which was chosen by using the snowball sampling method, they also used different methods for gathering the data for their research such as sociodemographic and obstetric data collection forms, pregnancy psychosocial health assessment scale, attitudes towards complementary and alternative medicine scale, and scale to measure superstition. The results of the study showed that the pregnant women's psychosocial levels were good and their levels of attitude to complementary and alternative medicine were moderate. To conclude, using the different methods of gathering data they found

that the personal and obstetric characteristics of pregnant women affected their psychosocial health status during pregnancy. They recommended that the factors to affect their psychosocial health status during pregnancy should be comprehensively evaluated and that pregnant women should be provided with necessary counseling and support.

Health-Seeking Behaviors

In an exploratory qualitative study conducted by Maritim et al. (2021), the researchers sought to explore the health beliefs and health-seeking behaviors of community members regarding Morbidity Management and Disability Prevention (MMDP) services for Lymphatic Filariasis (LF) in Luangwa District, Zambia. Lymphatic Filariasis (LF), also known as elephantiasis, is a parasitic disease caused by thread-like worms transmitted through mosquito bites. By utilizing four focus group discussions and twenty-six in-depth interviews for data collection and analyzing the data using thematic analysis, the researchers found that participants who attributed LF to witchcraft were more likely to seek treatment from traditional healers rather than healthcare specialists. However, a limitation of this study could be the small sample size, which might not fully represent the diversity of perspectives within Luangwa District, Zambia.

On the other hand, in a cross-sectional descriptive study conducted by Aranha et al. in 2022, they assessed the health-seeking behaviors of 236 undergraduate students pursuing courses in medicine, dentistry, nursing, and physiotherapy at a particular university. Using checklists and perception scales to collect data and analyzing the data

using descriptive and inferential statistics, the researchers found differences in the health-seeking behaviors of undergraduate students regarding physical and mental illnesses and the barriers they had to seek healthcare. Nonetheless, potential differences in the undergraduate students' demographics and cultural backgrounds, and limited generalizability may be present in the study.

Consequently, another descriptive cross-sectional study by Singh et al. in 2020 aimed to discover medical students' health-seeking behaviors at a teaching hospital in Nepal. A total of 235 students were included in their study, and a self-administered questionnaire was used. Furthermore, data entry and analysis were done using Statistical Package for Social Sciences version 20.0. In their study, the researchers found that although the medical students' behavior toward visiting medical facilities was favorable, the first people they consulted before a health professional were their parents. Additionally, common barriers among medical students included their perception of health issues as minor and a lack of time for check-ups. Likewise, the study may have limitations in generalizability as the respondents were only among the first and second-year students at a particular teaching hospital in Nepal.

Health Literacy

Cuthino et al. (2021) conducted a comprehensive study assessing health literacy among health science members and non-health science members, in which they utilized a methodology of a quantitative questionnaire implementing the stratified proportionate sampling among 230 samples. Studies have shown that individuals in the health members have lower health literacy compared to non-health science members.

Following the results of Cuthino et al. (2021), it is indicated that there are major gaps in the health literacy of non-health professionals. Additionally, healthcare facilities may contribute to improving health literacy levels by fostering more effective interaction methods, resulting in enhanced medical results and improved interpersonal interaction among healthcare professionals and patients.

Similarly, Šulinskaitė et al. (2022) discovered a correlation between low health literacy levels and specific demographic characteristics, such as older age, urban location, lower education, and being single. By surveying European residents with a focus on younger patients, living in urban areas, having higher education, and living with a partner, the study revealed the prevalence of gaps in health literacy and identified specific demographic segments that are particularly vulnerable to low levels of health literacy.

STATEMENT OF THE PROBLEM

This study aims to determine the influence of health superstitions on the health-seeking behaviors of Ateneo de Zamboanga Senior High School Students.

Specifically, it seeks to answer the following research questions:

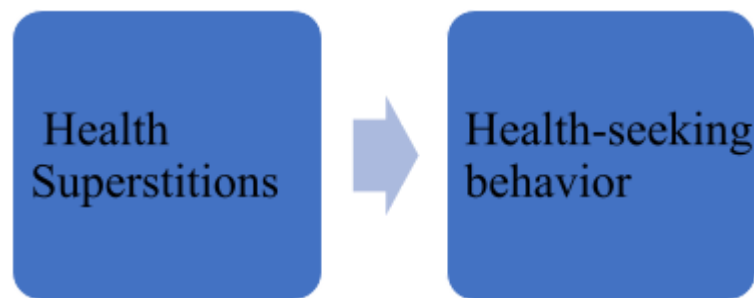
1. What is the health superstition level of the students?
2. What are the health-seeking behaviors of the students?
3. How do health superstitions influence the health-seeking behaviors of the students?

From the questions above, the following hypotheses were formulated:

H0: There is no significant correlation between the health superstitions and health-seeking behavior of Ateneo de Zamboanga University senior high school students.

H1: There is a significant correlation between the health superstitions and health-seeking behaviors of Ateneo de Zamboanga University senior high school students.

CONCEPTUAL FRAMEWORK



The conceptual framework of this study illustrates the relationship between health superstitions and health-seeking behaviors among students. It suggests that health superstitions, which are beliefs or practices not rooted in scientific evidence, can directly influence the way students approach health-related issues. The framework posits that these superstitions may shape students' decisions when it comes to seeking medical help, possibly leading them to rely on traditional or non-conventional methods rather than evidence-based healthcare practices. By examining this connection, the study seeks to understand how these beliefs affect students' health behavior patterns and decision-making processes.

SIGNIFICANCE OF THE STUDY

This study underscores the importance of exploring the influence of health superstitions on health-seeking behaviors of Ateneo de Zamboanga University senior high school students, offering a deeper understanding of its broader implications. The findings aim to enhance current perspectives and foster future advancements in the area of interest.

ATENEO DE ZAMBOANGA ADMINISTRATION. The findings of this study will be of help to the Ateneo de Zamboanga University Administration because, with it, they will be able to gain insights regarding the extent of influence the AdZU senior high students' health superstitions have on their health-seeking behaviors and, in turn, provide enhancements to their students. Following this, the administration can plan and implement programs or gathering events that would assist the students in not only improving their attitudes and perceptions about health, health superstitions, and health practices but also encourage them to seek medical help whenever they perceive themselves to feel ill.

ATENEO DE ZAMBOANGA UNIVERSITY INFIRMARY. The infirmery employees or nurses can provide valuable insights. Since the employees/nurses in the infirmery have knowledge related to health, they can assist students with their concerns and correct misinformation. Additionally, they often have direct interactions with other students who experience health superstitions. Because of this, they can provide an understanding of how health superstitions work and their relationship to health.

STUDENTS AND THEIR FAMILIES. This study will serve as a vital tool in

illuminating the health superstitions and health-seeking behaviors prevalent among the students of Ateneo de Zamboanga Senior High School and their families. By delving into these cultural beliefs and practices surrounding health and wellness, the research aims to provide a comprehensive understanding of how individuals perceive and engage with healthcare services. By identifying common superstitions, misconceptions, and preferred methods of seeking medical assistance, the study will provide critical information about the influence of health superstitions on the health-seeking behaviors of the students.

HEALTH PROFESSIONALS. The findings of the study will serve as a cornerstone for health professionals, offering nuanced insights into the intricate interplay between health superstitions and health-seeking behaviors, and how they mutually influence one another. By unraveling the complex web of beliefs, attitudes, and practices surrounding health and wellness within the community of Ateneo de Zamboanga Senior High School students, the research will provide health professionals with a deeper understanding of the contextual factors shaping their patients' healthcare decisions. Armed with this knowledge, healthcare providers will be better equipped to engage with their patients in culturally sensitive and empathetic ways, fostering trust, communication, and collaboration in the pursuit of improved health outcomes. Moreover, the insights from the study will inform the development of tailored interventions, patient education materials, and support systems that resonate with the community's beliefs and preferences, ultimately enhancing the effectiveness and relevance of healthcare delivery in addressing the diverse needs of their patients.

SCOPE AND LIMITATIONS

This study aims to address a significant gap in the existing research by delving into the demographic of Ateneo de Zamboanga Senior High School students, with the primary objective of uncovering and interpreting the influence of health superstitions on the health-seeking behaviors of the students. The research will be conducted within the context of Ateneo de Zamboanga University (AdZU), focusing on Senior High School students enrolled during the academic year 2024-2025. By examining this specific population, the study seeks to gain insights into how health superstitions shape students' decisions regarding health care. However, the study is limited to the AdZU Senior High School demographic, meaning its findings may not be applicable to other universities, younger age groups, or different cultural contexts. Additionally, the research will rely on self-reported data, which may introduce biases such as social desirability or recall bias. The timeframe of the study may also limit the scope of data collection, as only a brief view of the students' health-seeking behaviors and beliefs during a specific period will be analyzed.

DEFINITION OF TERMS

The following terms are defined operationally:

- a. **Ateneo de Zamboanga Senior High School** - It refers to an esteemed educational institution situated in Zamboanga City, Philippines. As part of the Ateneo de Zamboanga University system, it offers a comprehensive secondary

level of education, encompassing a wide range of academic disciplines and extracurricular activities. With a focus on holistic development, the school nurtures students not only academically but also instills values of integrity, leadership, and social responsibility, preparing them to become well-rounded individuals ready to contribute positively to society.

- b. **Ateneo de Zamboanga Senior High School students** - This refers to individuals enrolled in the 11th and 12th grades within the ABM, STEM, and HUMSS strands for the academic year 2024-2025 at Ateneo de Zamboanga University Senior High School.
- c. **Beliefs** - This refers to strong ideas or thoughts that people have about what is true or real. Beliefs can encompass a wide range of ideas, including religious, cultural, political, philosophical, or personal convictions. They are often deeply ingrained and influence how individuals interpret the world around them and make decisions. Beliefs can be based on personal experiences, teachings from authority figures, cultural norms, or inherited traditions. They play a significant role in shaping identity and can vary greatly from person to person or from one culture to another. Additionally, beliefs can evolve over time due to new experiences, education, or exposure to different perspectives.
- d. **Health** - In this study, Health refers to the comprehensive condition of an individual's physical, mental, and social well-being along with an emphasis on the absence of disability or illness of Ateneo de Zamboanga Senior High School students.

- e. **Health-Seeking Behavior** - In this study, it refers to how the Ateneo de Zamboanga Senior High School students seek guidance and support regarding their health with the assistance of AdZU employees, this will empower the AdZU students to make informed decisions and accept responsibility for their well-being.
- f. **Health Superstitions** - In this study, it refers to beliefs and practices concerning the perceptions of AdZU SHS students regarding their health, what constitutes it, the causes of illness, and approaches to overcoming it, which are not supported by scientific reasoning.
- g. **Superstitions** - These are beliefs about particular events that scientific reasoning can hardly interpret. These beliefs can be religious, cultural, and personal beliefs. In this study, the influence of superstitions regarding health is the primary focus of understanding the health-seeking behavior of Ateneo de Zamboanga Senior High School students.
- h. **Traditional and Complementary Medicine (T&CM)** - It refers to the holistic way of treating health issues with the use of information, skills, and practices built around theories, beliefs, and experiences across diverse cultures—as well as the wide range of alternative medical procedures that are not part of a particular nation’s healthcare system, medical traditions or mainstream medicine, yet more often than not, are more accessible and more affordable.

CHAPTER TWO

METHODOLOGY

This chapter discusses the research methodology used in the study, including the research design, data collection methods, and analysis procedures. The study employs a quantitative research design, utilizing surveys as the primary data collection method. The target population and sampling technique are carefully selected to ensure the reliability and accuracy of the results. The research instrument consists of a structured questionnaire designed to gather relevant data from the respondents. The survey is distributed systematically, ensuring that responses accurately reflect the influence and relationship between health superstitions and health-seeking behaviors. The collected data is then analyzed using appropriate statistical tools to identify patterns and relationships between the variables. Additionally, ethical considerations are observed throughout the study, including obtaining informed consent from participants and ensuring the confidentiality of their responses. Lastly, the limitations of the study are acknowledged, recognizing any factors that may affect the generalizability of the findings.

RESEARCH DESIGN

The study will use a quantitative research approach. Specifically, the study will utilize a causal research design. Causal research design, or explanatory research, refers to investigating the nature and extent of a cause-and-effect relationship between independent and dependent variables of a phenomenon (*Causal Research (Explanatory*

Research), n.d.). Consequently, this study aims to establish whether health superstitions influence health-seeking behaviors among the Ateneo de Zamboanga senior high school students. Thus, a causal research design will be adequate for this research to provide valuable insights regarding the presence or absence of a cause-and-effect relationship between the Ateneo de Zamboanga senior high school students' health-seeking behaviors and their health superstitions. Additionally, if there is indeed a relationship between health superstitions and health-seeking behaviors among the Ateneo de Zamboanga senior high school students, a causal research design will be sufficient in determining the extent to which the health superstitions may influence the students' health-seeking behaviors.

RESEARCH LOCALE

The study will be conducted at Ateneo de Zamboanga University, La Purisima Street, Zamboanga City, as the study will involve both students and faculty members. The researchers selected this university as the location for their study for some reasons. Firstly, AdZU has a diverse population of students and faculty members, making it an ideal sample for research. Secondly, universities are known to be a conducive environment for studying health issues, and both students and faculty members may encounter various health challenges. Lastly, AdZU's location is easily accessible to the researchers, guaranteeing their easy collection of data and engagement with their select participants. Overall, this setting will provide opportunities in the exploration of health literacy issues within a relevant and accessible context.

RESEARCH PARTICIPANTS/SUBJECT

The study will involve Senior High School students enrolled in Ateneo de Zamboanga University (AdZU). The researchers will utilize cluster sampling to select 160 participants from a student population of 900. The sample group will be randomly chosen from five class sections, each representing students from STEM, HUMSS, and ABM tracks. This sampling method will provide efficiency in conducting the survey process.

Since the study will be conducted within the institution, it is accessible, making it easier for researchers to reach out. Additionally, through this research topic, participants will have the opportunity to contribute their insights and experiences, ensuring their active involvement and alignment with the study's objectives. The research team will also prioritize considerations to ensure the confidentiality of information. The steps to be performed will be put in place to ensure a protected setting for the students to engage in the research study.

SAMPLING DESIGN/SUBJECT COLLECTION

The research sampling that will be used in this study is simple random sampling. Simple random sampling involves selecting a random selection of individuals from a larger group or population (Horton 2024). Employing simple random sampling, senior high school students of Ateneo de Zamboanga University will be selected randomly from the entire student population without considering their sections or academic tracks. The researchers will inform the respondents about the study personally and then distribute assent forms for students and parental consent forms for their parents. Simple

random sampling is typically used when a researcher wants to select a sample from a population without bias (Horton, 2024). The researchers chose this sampling method as it is deemed suitable to the study's wide population and is logistically easier to implement. Simple random sampling is relatively simple compared to other methods because there are no complex steps or special skills required. The researchers chose simple random sampling as it is also cost-effective. This method ensures that each individual in the population has an equal chance of being selected, which eliminates the need for dividing groups into clusters or strata. As a result, it simplifies the sampling process, making it easier to execute while still providing reliable results. Additionally, it requires little to no prior knowledge about the population, reducing the time and effort needed for planning and coordination. The inclusion criteria will encompass Ateneo de Zamboanga University senior high school students currently enrolled in grades 11 and 12 who can complete the assent and obtain parental permissions through the parent's consent form provided by the researchers. Additionally, students must be present and able to participate during the study period, willing to report health-seeking behaviors and beliefs honestly, and proficient in language used for data collection to ensure accurate responses. These criteria ensure the sample consists of eligible students who are likely to provide meaningful data for the research objectives, enhancing validity and applicability. The exclusion criteria include students enrolled in other educational institutions or not attending Ateneo de Zamboanga University Senior High School, as well as those lacking consent from parents or legal guardians. These criteria aim to refine the sample to active Ateneo de Zamboanga University Senior High School students likely to provide reliable data for the study.

DATA GATHERING PROCEDURES AND INSTRUMENTS

The data for this study were collected through a structured questionnaire distributed to 118 Senior High School students enrolled at Ateneo de Zamboanga University (AdZU) during the 2024-2025 academic year. The questionnaire was designed to gather information on students' demographic characteristics, beliefs in health superstitions, and health-seeking behaviors.

The survey was administered both in person and online to maximize participation. For in-person distribution, data collection took place during break hours to ensure a high response rate. For online distribution, a Google Forms link was sent to participants via Messenger. Prior to participation, informed consent was obtained from both students and their parents or guardians (for minors) to ensure ethical compliance. In the online survey, consent was administered through a check box. Students were assured of the confidentiality of their responses and the voluntary nature of their participation.

The questionnaire consisted of three sections. The first section collected demographic information, including age, gender, and grade level. The second section assessed students' beliefs in health superstitions, while the third section focused on their health-seeking behaviors, such as their likelihood of seeking professional medical care versus relying on alternative treatments. To ensure clarity and accuracy, the questionnaire was pre-tested with a small group of students before full distribution.

The primary data collection tool was a self-administered questionnaire designed specifically for this study. It consisted of closed-ended questions using dichotomous and

frequency scales to measure responses effectively. The materials also included informed consent forms, which were provided to both students and parents or guardians.

DATA ANALYSIS

For data analysis, the responses were inputted and organized into an Excel spreadsheet and then coded for further interpretation. The data for the health superstitions and health-seeking behaviors were computed by percentage according to the frequencies of responses for every option. Consequently, Spearman's rho correlation — a non-parametric correlation measurement that “assesses the strength and direction of the relationship between two variables” by data point ranking (Sazonets et al., 2024) — was utilized to find out the relationship between health superstitions and health-seeking behaviors, while the Correlation Coefficient interpretation table by Emmanuel and Isiaq (2024) was used to determine the strength of relationship between health superstitions and the students health-seeking behaviors.

ETHICAL CONSIDERATIONS

To ensure the protection of participants' rights and the confidentiality of participants' information, the researchers will first obtain approval for the study from their research teacher, representing the research department of the institution. Once approved, the researchers will provide an oral explanation to the participants regarding the main purpose of the research, the approximate time required, and a full description of the benefits and risks associated with participating in the survey. This explanation will also include details on how confidentiality will be maintained throughout the study.

Following this, the researchers will request signed assent from participants under the age of eighteen and assent from those above eighteen, along with parental consent. To safeguard the personal data of participants, the results of the survey will remain confidential and will not be disclosed to anyone other than the researchers involved in the study. However, there may be a psychological risk associated with the study, as some questions may be sensitive for participants. To address this concern, participants will be assured of their voluntary participation and their right to stop answering the survey at any time. They will also be informed that they can contact the researchers if they do not want their personal data to be used. Despite the potential risks, the benefits of the study outweigh them. The success of this research may help participants identify their level of superstition beliefs and determine their health-seeking behavior, thereby contributing to a better understanding of the relationship between the two.

CHAPTER THREE

RESULTS AND DISCUSSION

This chapter presents the findings on the influence of health superstitions on the health-seeking behaviors of the senior high school students of Ateneo de Zamboanga University. The findings are summarized in tables, showing trends, frequencies, and significant correlations. The results are then interpreted by comparing them with previous studies, highlighting patterns and differences. Additionally, the implications of the findings are discussed in relation to public health, emphasizing how superstitious beliefs may influence medical decisions.

DEMOGRAPHIC

The study involved 118 Senior High School students from Ateneo de Zamboanga University (AdZU), with participants ranging in age from 16 to 18 years. The majority of respondents (79.8%) were female and 20.2% were male. In terms of grade level, 66% of the participants were in Grade 12, and 34% were in Grade 11. This demographic profile provides context for understanding the diversity of beliefs and behaviors among the students, which may influence their health-seeking behaviors and the role that health superstitions play in their decision-making processes.

RESEARCH QUESTION NO. 1 : WHAT IS THE HEALTH SUPERSTITION LEVEL OF THE STUDENTS?

Table 1

Health superstitions (n=118)

Health-belief	Agree	Disagree
----------------------	--------------	-----------------

Superstitions	(%)	(%)
Get rid of a hiccup by placing a short thread wet with saliva on the forehead.	11.86	88.14
By showing fondness or affection when meeting a baby for the first time, you may cause the baby to feel uneasy or make the baby cry non-stop which is commonly referred to as “na-usog.”	39.83	61.16
Sleeping right after a shower is believed to cause blindness and insanity.	17.80	82.20
Washing sweaty hands can lead to spasmodic hands or pasma.	38.98	61.02
Not patting your sweat dry with a towel can cause you to get pneumonia.	45.76	54.24
Stepping over a child will stunt his or her growth	49.15	50.85
Frog urine causes warts.	19.50	80.50
Showering salt over the threshold of the front door of your new house can help you and your family ward off sickness.	27.12	72.88
Hopping over stair steps to keep menstrual period short.	24.58	75.42
Sucking on your pricked finger so blood re-enters your body.	30.51	69.49

Table 1 presents students' beliefs regarding various health superstitions. The majority of senior high school students disagreed with these beliefs, with 88.14% rejecting the idea that placing a wet thread on the forehead cures hiccups and 82.20% dismissing the notion that sleeping after a shower causes blindness. However, some superstitions received mixed responses, such as the belief that stepping over a child

stunts their growth (49.15% agreement) and the concept of na-usog (39.83% agreement). Overall, 69.59% of students disagreed with health superstitions, indicating that most do not adhere to these beliefs, resulting in a generally low level of superstition. These findings contrast with those of Guzman et al. (2021), who reported that cultural practices and superstitions significantly influence students' health perceptions, often leading to reliance on alternative remedies. Moreover, Fernández et al. (2024) found that Filipino students frequently blend traditional beliefs with modern medical practices, particularly among lower-income groups.

In contrast to the findings of Uwayezu et al. (2022), superstitions can lead to dangerous health practices. Certain superstitions, such as beliefs about infants born with erupted teeth, have resulted in harmful practices, including infanticide. Studies have highlighted the dangers of FGM, showing high rates of complications such as excessive bleeding and infections. In contrast, male circumcision, when performed in a clinical setting, has proven medical benefits, though traditional circumcision in rural areas can pose risks.

RESEARCH QUESTION NO. 2 : WHAT ARE THE HEALTH-SEEKING BEHAVIORS OF THE STUDENTS?

Table 2

Health-seeking behaviors (n=118)

Indicators	Options	Frequency	Percentage (%)
Frequency of visiting doctor for regular check-ups	Once a year	8	6.8
	Every few months	9	7.6
	Every 6 months	4	3.4
	Only when I feel unwell	87	73.7
	Never	10	8.5
If feeling unwell , would you go to the university's infirmary?	Yes	85	72.
	No	33	28.
Do you take any medication?	Yes	23	19.5
	No	95	80.5
To whom do you usually seek health advice when you feel unwell?	Healthcare professionals	49	41.5
	Family and friends	54	45.8
	Online resources	15	12.7
	Traditional or alternative practitioners	0	0
What is your first course of action when you experience mild symptoms like a headache or a cold?	Visit the doctor or other health facilities like infirmary or health centers	13	11
	Wait for the symptom to subside	35	29.7
	Seek home remedies or traditional medicine	70	59.3

Do you engage in preventive health measures (e.g., vaccinations, health screenings)?	Never	4	3.4
	Rarely	13	15.3
	Sometimes	38	32.2
	Often	31	26.3
	Always	27	22.9
Do you perform actions that are not known to be scientifically proven	Never	11	9.3
	Rarely	18	15.3
	Sometimes	46	39
	Often	31	26.3
	Always	12	10.2
How often do you search for health information using the Internet?	Never	4	3.4
	Rarely	13	11
	Sometimes	27	22.9
	Often	35	29.7
	Always	39	33.1
Do you take further action after researching health information on the internet?	Never	17	14.4
	Rarely	22	18.6
	Sometimes	47	39.8
	Often	19	16.1
	Always	13	11

As shown in Table 2, the health-seeking behaviors of AdZU senior high school students indicate that a significant majority visit a doctor for regular check-ups only when they feel unwell (73.7%) . Meanwhile, 72% would choose to go to the university's infirmary when feeling unwell, but 28% would not. Regarding medication i.e. maintenance 80.5% do not take any, whereas 19.5% do. For health advice, 45.8% primarily consult family and friends, while 41.5% seek guidance from healthcare professionals and none reported visiting traditional or alternative practitioners. When

experiencing mild symptoms such as headaches or colds, 59.3% prefer home remedies or traditional medicine, 29.7% wait for the symptoms to pass, and only 11% seek help from a doctor or health facility.

Moreover, preventive health measures, including vaccinations and health screenings, are consistently practiced by 22.9% of students and often by 26.3%, while 32.2% sometimes engage in these activities. Meanwhile, actions that lack scientific evidence are sometimes taken by 39% of students, often by 26.3%, and always by 22.9%. Using the Internet for health information is prominent, with 33.1% of the students always searching online, 29.7% often, and 22.9% sometimes. After researching health information, 39.8% sometimes take further action, 16.1% often, and 11% always while 14.4% reported never.

The findings indicate that most students seek medical care only when they perceive it as necessary, reflecting a reactive approach to health. This aligns with Mboweni and Sumbane (2019), who found that adolescents in North West Province, South Africa, delay seeking healthcare until complications arise due to factors such as negative attitudes from healthcare providers, peer pressure, lack of weekend services, long queues, distance, and insufficient health knowledge. Regular check-ups play a crucial role in early detection and intervention, reducing the need for extensive medical treatment (Pomona Valley Health Centers, 2023). In line with this, Agraan et al. (2024) utilized the Adolescent Health and Development Program, a comprehensive program aims to promote healthy lifestyles and relationships among adolescent, to assess its impact on 250 Grade 11 and 12 students at the Special Health Sciences Senior High School of the De La Salle Medical and Health Sciences Institute. The study found that

the program positively influenced students' health behaviors, promoting better awareness of contraception, mental health support, and preventive healthcare practices.

On the other hand, many students prefer to manage minor illnesses with home remedies rather than seek professional care. The findings show that 59.3% of students rely on home remedies or traditional medicine for mild symptoms such as headaches or colds, while 29.7% choose to wait for symptoms to pass, and only 11% seek medical attention. This highlights a tendency toward self-care and a reluctance to consult healthcare professionals for minor ailments. The present findings also align with those of Bhandari & Chataut (2020), who reported that 28% of respondents avoided hospital visits due to perceiving their illnesses as minor, while 12% specifically relied on home remedies. Similarly, Sheikh et al. (2022) found that students prefer home remedies due to their accessibility, perceived safety, and effectiveness. This pattern is particularly evident among adolescent girls, who often use home remedies to manage menstrual pain, such as dysmenorrhea.

These results indicate that students are more likely to seek medical help when they consider their symptoms severe, suggesting a direct link between symptom perception and health-seeking behaviors. Supporting this, Gunarathna (2023) found that students with higher perceived severity of health problems are more inclined to actively seek health information and take action.

The data shows that social support plays a crucial role in health decision-making of the students evident in 45.8% primarily choosing to consult family and friends for health advice, while 41.5% seek guidance from healthcare professionals. This is consistent with Gunarathna (2023), who reported that approximately 41% of students

rely on informal networks for health advice, sometimes leading to engagement in health practices lacking scientific evidence. When further information is needed, students frequently turn to the Internet, with 46.2% favoring online resources for health-related concerns. This data highlights the increasing influence of digital platforms on students' health-seeking behaviors.

RESEARCH QUESTION NO. 3 : HOW DO HEALTH SUPERSTITIONS INFLUENCE THE HEALTH-SEEKING BEHAVIORS OF THE STUDENTS?

Spearman rho correlations were computed to examine the relationship between the health superstition level and health-seeking behaviors of AdZU senior high school students. The results are summarized in Table 3 below.

Table 3

Spearman rho correlation between health superstitions and health-seeking behaviors (r=117)

Indicators	Spearman's rho	p-value
How often do you visit a doctor for regular check-ups?	0.083	0.367
If feeling unwell, would you go to the university's infirmary?	-0.052	0.577
Do you take any medication regularly? (e.g., insulin, allergy)	0.035	0.708
To whom do you usually seek health advice when you feel unwell	-0.187*	0.042
What is your FIRST course of action when you experience mild symptoms?	0.142	0.123
Do you engage in preventive health measures (e.g., vaccinations, health screenings)?	0.128	0.165

Do you perform actions that are NOT KNOWN to be scientifically proven (e.g., taking a shower after getting soaked in the rain to prevent having a cold or fever)?	0.309***	< .001
How often do you search for health information using the Internet? (for example, if you were experiencing unfamiliar symptoms, would you look online to find out what it could be?)	0.155	0.093
Do you take further action after researching health information?	0.09	0.333

Note. * $p < .05$, ** $p < .01$, *** $p < .001$

Upon using the p-value, $p < .05$, as the basis to determine whether there is a significant relationship between health superstitions and health-seeking behaviors, Table 3 shows a majority of the indicators having a higher p-value, which meant that the relationship between the health superstition level and health-seeking behaviors of AdZU senior high school students showed no significance. Thus, the frequency of the students' regular check-up with a doctor has no significant relationship with their health superstition level, $r(117) = 0.083$, $p = 0.367$. The students' frequency of visiting the university's infirmary when feeling unwell has no significant relationship with their health superstition level, $r(117) = -0.052$, $p = 0.577$. The frequency of student's regular intake of medication has no significant relationship with their health superstition level, $r(117) = 0.035$, $p = 0.708$. The first course of action the student's take when experiencing mild symptoms has no significant relationship with their health superstition level, $r(117) = 0.142$, $p = 0.123$. The students' engagement in preventive health measures has no significant relationship with their health superstition level, $r(117) = 0.128$, $p = 0.165$. The frequency of students using the Internet to search for health information has no significant relationship with their health superstition level, $r(117) = 0.155$, $p = 0.093$. The

students' actions after researching health information online has no significant relationship with their health superstition level, $r(117) = 0.09$, $p = 0.333$.

Regardless, several indicators resulted in a p-value lower than the basis, $p < 0.05$, and revealed a significant relationship between the health superstition level and health-seeking behaviors of AdZU senior high school students. Hence, the resources students turn to for health advice when feeling unwell showed a significant relationship with their health superstition level, $r(117) = -0.187$, $p = 0.042$. Likewise, the inclination of students to perform actions that are not known to be scientifically proven also showed a significant relationship with the students' health superstition level, $r(117) = 0.309$, $p < 0.001$.

Utilizing the Correlation Coefficient interpretation table by Emmanuel & Isiaq (2024), the correlation strength of the indicators identified to have a significant correlation between the health superstition level and health-seeking behaviors of the students were determined. Upon observing the Spearman's rho, the resources students turn to for health advice when feeling unwell was interpreted to have a weak negative correlation, which signifies that the higher the health superstition level of the students, the higher chance they seek help from their families and friends and online resources rather than healthcare professionals. This finding is similar to the study of Gunarathna (2023) on the Health Information Seeking Behavior (HSIB) of Sri Lanka university students, wherein 46.2% of them use the Internet as their primary source of health information and 41% depend on interpersonal networks while only 13% consult healthcare professionals. Students tend to rely more on potentially unreliable sources than the experts.

Furthermore, the inclination of students to perform actions that are not known to be scientifically proven was interpreted to have a moderate positive correlation, which signifies that the higher the superstition level of the students, the higher chance that they perform actions that are not known to be scientifically proven. In relation to this, a study by Imran et al. (2024) identified various themes related to superstitions among educated urban populations in Pakistan, indicating that superstitious beliefs can still prevail among educated individuals. In response to findings such as these, Uwayezu et al. (2022) proposed that increasing health literacy and scientific understanding can combat practices driven by superstition.

CHAPTER FOUR

CONCLUSION AND RECOMMENDATION

This chapter presents the conclusions and recommendations drawn from the results of the study, summarizing key findings on the influence between health superstitions and health-seeking behaviors of the senior high school students of Ateneo de Zamboanga University.

CONCLUSIONS

Research Question No. 1: What is the health superstition level of the students?

The study found that the majority of AdZU Senior High School students exhibit generally low belief in health superstitions. This suggests that while superstitious beliefs may still exist among some individuals, they are not a dominant factor in their health-seeking behavior. The low level of superstition implies that most students rely more on scientific reasoning and practicality when making health-related decisions.

Research Question No. 2: What are the health-seeking behaviors of the students?

The health-seeking behaviors of the students indicate a tendency to seek medical attention only when symptoms become concerning. Many prefer home remedies or advice from family and friends rather than consulting healthcare professionals for minor ailments. Additionally, the Internet serves as a major source of health information, which, while convenient, may expose students to misinformation or unreliable sources.

Research Question No. 3: How do health superstitions influence the health-seeking behaviors of the students?

The study found that while health superstitions do not have a dominant influence on the health-seeking behaviors of AdZU Senior High School students, there is still a significant correlation between superstition levels and certain behaviors. Students with higher superstition levels are more likely to seek health advice from non-medical sources such as family, friends, and the Internet rather than from healthcare professionals. Additionally, they are more inclined to engage in health-related actions that lack scientific basis. This influence highlights the importance of strengthening health literacy and promoting critical thinking to ensure students make informed and evidence-based health decisions.

RECOMMENDATIONS

The following recommendations are based on the findings of this study to address the influence of health superstitions on health-seeking behaviors. These suggestions aim to encourage more informed health choices and improve students' approach to seeking professional medical care.

- 1. Ateneo de Zamboanga University Administration.** The Ateneo de Zamboanga University Administration should consider collaborating with the Red Cross Youth Council (RCYC) in conducting general symposiums or comprehensive programs that aim to educate senior high school students on the significance of health literacy and seeking medical aid in fostering healthy lifestyles. Otherwise, the administration can include the importance of scientific understanding as a learning

competency in Homeroom Guidance (HG) sessions for the moderators to research upon the topic and teach their students accordingly.

2. **Ateneo de Zamboanga University Infirmary.** The Ateneo de Zamboanga University Infirmary can administer regular medical check-ups among senior high school students to reduce the students' chances of self-diagnosing via information found in the Internet and as a preventive health measure to detect developing symptoms for the students to proactively approach their health issues by seeking aid from healthcare professionals.
3. **Students and their families.** Students and their families are advised to become proactive rather than reactive in caring for their health by engaging in preventive health measures such as vaccinations and health screenings for early detection and intervention as well as reduced urgency for extensive medical treatments. Additionally, students and their families are recommended to consult health professionals when feeling symptoms instead of self-diagnosing through potentially unreliable health information online or performing traditional treatments that can risk rather than improve their health.
4. **Health professionals.** Health professionals can publish scientific literature that debunk common Filipino health superstitions and further elaborate on their implications to health, especially when traditional medicine or treatments get involved.

APPENDICES

APPENDIX A: Research Instrument

Name (optional) :
Strand, Grade Level, & Section :
Gender:

Do you believe in any of these health-related superstitions?

Health-Related Superstitions	Agree	Disagree
Get rid of a hiccup by placing a short thread wet with saliva on the forehead.		
By showing fondness or affection when meeting a baby for the first time, you may cause the baby to feel uneasy or make the baby cry non-stop which is commonly referred to as "na-usog."		
Sleeping right after a shower is believed to cause blindness and insanity.		
Washing sweaty hands can lead to spasmodic hands or pasma.		
Not patting your sweat dry with a towel can cause you to get pneumonia.		
Stepping over a child will stunt his or her growth.		
Frog urine causes warts.		
Showering salt over the threshold of the front door of your new house can help you and your family ward off sickness.		
Hopping over stair steps to keep menstrual period short		
Sucking on your pricked finger so blood re-enters your body		

HEALTH-SEEKING BEHAVIORS

- How often do you visit a doctor for regular check-ups?
 - Once a year
 - Every few months
 - Every 6 months
 - Only when I feel unwell
 - Never
- If feeling unwell would you go to the university's infirmary?
 - Yes
 - No
- Do you take any medication regularly? (e.g. insulin, allergy medication, etc.)
 - Yes
 - No
- To whom do you usually seek health advice when you feel unwell?
 - Healthcare professionals (doctors, nurses, etc.)
 - Family and friends
 - Online resources (websites, apps, forums)
 - Traditional or alternative practitioners

5. What is your FIRST course of action when you experience mild symptoms like a headache or a cold?

- Visit the doctor or other health facilities like infirmary or health centers
- Wait for the symptom to subside
- Seek home remedies or traditional medicine

6. Do you engage in preventive health measures (e.g., vaccinations, health screenings)?

- Never
- Rarely
- Sometimes
- Often
- Always

7. Do you perform actions that are NOT KNOWN to be scientifically proven (e.g., taking a shower after getting soaked in the rain to prevent having a cold or fever)?

- Never
- Rarely
- Sometimes
- Often
- Always

8. How often do you search for health information using the Internet? (for example, if you were experiencing unfamiliar symptoms, would you look online to find out what it could be?)

- Never
- Rarely
- Sometimes
- Often
- Always

9. Do you take further action after researching health information on the internet?

- Never
- Rarely
- Sometimes
- Often
- Always

Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students

Dear Participant,

We are Aisha Abdula, Kimberly Hamisani, Yuzra Jumail, and Abegail Mahilum, grade 12 students from Science, Technology, and Mathematics (STEM) strand of Ateneo de Zamboanga University Senior High School. We are conducting a quantitative research study titled "Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students." We are writing to invite you to participate as a valued respondent in our study.

Our study aims to **assess the health superstition level and health-seeking behaviors** among the Ateneo de Zamboanga Senior High School students. As a student, you may have encountered or observed certain beliefs or practices related to health that might be based on superstitions but are not scientifically proven.

By gathering information from diverse participants, we aim to gain profound knowledge and understanding of these dynamics and their impact on students' health care practices. **Your valuable contribution will provide essential data and enhance our understanding of how health superstitions affect health-seeking behaviors among students.** We assure you that your participation is vital in enriching our research.

To ensure your comfort and convenience, we have designed the study procedure to involve a survey questionnaire that will be answered through this google form. **Participation in this data gathering is completely voluntary. Your decision and response will be respected and treated with utmost confidentiality.**

Although the study is designed to reduce any possible risks or discomforts, it is important to note that some questions may bring discomfort or cause you to be hesitant when answering the questionnaire, especially those questions related to health-seeking behaviors.

Nonetheless, **the researchers will ensure privacy and confidentiality when handling your data.** We want to be transparent about this possibility and assure you that your comfort and privacy are our top priorities.

Name (LN, FN, MI) *

Your answer

 This is a required question

Grade and Section *

Choose

Gender *

☐ Male

☐ Female

CONSENT *

☒ I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Belief in Health-related Superstitions

	Agree	Disagree
Get rid of a hiccup by placing a short thread wet with saliva on the forehead.	☐	☐
By showing fondness or affection when meeting a baby for the first time, you may cause the baby to feel uneasy or make the baby cry non-stop which is commonly referred to as "na-usog."	☐	☐
Sleeping right after a shower is believed to cause blindness and insanity.	☐	☐
Washing sweaty hands can lead to spasmodic hands or pasma.	☐	☐
Not patting your sweat dry with a towel can cause you to get pneumonia.	☐	☐
Stepping over a child will stunt his or her growth	☐	☐
Frog urine causes warts.	☐	☐
Showering salt over the threshold of the front door of your new house can help you and your family ward off sickness.	☐	☐
Hopping over stair steps to keep menstrual period short.	☐	☐
Sucking on your pricked finger so blood re-enters your body.	☐	☐

Back

Next

Clear form

Never submit passwords through Google Forms.

This form was created inside of Ateneo de Zamboanga University.
Does this form look suspicious? [Report](#)

Google Forms

Health-Seeking Behaviors

1. How often do you visit a doctor for regular check-ups?

- ☐ Once a year
☐ Every few months
☐ Every 6 months
☐ Only when I feel unwell
☐ Never

2. If feeling unwell would you go to the university's infirmary?

- ☐ Yes
☐ No

3. Do you take any medication regularly? (e.g. insulin, allergy medication, etc.)

- ☐ Yes
☐ No

4. To whom do you usually seek health advice when you feel unwell?

- ☐ Healthcare professionals (doctors, nurses, etc.)
☐ Family and friends
☐ Online resources (websites, apps, forums)
☐ Traditional or alternative practitioners

5. What is your FIRST course of action when you experience mild symptoms like a headache or a cold?

- ☐ Visit the doctor or other health facilities like infirmary or health centers
☐ Wait for the symptom to subside
☐ Seek home remedies or traditional medicine

6. Do you engage in preventive health measures (e.g., vaccinations, health screenings)?

☐ Never
☐ Rarely
☐ Sometimes
☐ Often
☐ Always

7. Do you perform actions that are NOT KNOWN to be scientifically proven (e.g., taking a shower after getting soaked in the rain to prevent having a cold or fever)?

☐ Never
☐ Rarely
☐ Sometimes
☐ Often
☐ Always

8. How often do you search for health information using the Internet? (for example, if you were experiencing unfamiliar symptoms, would you look online to find out what it could be?)

☐ Never
☐ Rarely
☐ Sometimes
☐ Often
☐ Always

9. Do you take further action after researching health information on the Internet?

☐ Never
☐ Rarely
☐ Sometimes
☐ Often
☐ Always

Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students

sh230532@adzu.edu.ph [Switch account](#) 

 Not shared

Thank you for your valuable input in our research study!

If you have any question, concern or any clarification, do not hesitate to contact us at **sh230656@adzu.edu.ph**. It will be our pleasure to provide you with further information.

[Back](#)
[Submit](#)
[Clear form](#)

APPENDIX B: Informed Consent



Ateneo de Zamboanga University
SENIOR HIGH SCHOOL
RESEARCH DEPARTMENT
La Purisima Street, Zamboanga City 7000 | (062)991-0871 local 3050



INVITATION LETTER

“Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students”

Dear Participant,

We are **AISHA ABDUA, KIMBERLY HAMISANI, YUZRA JUMLAIL, and ABEGAIL MAHILUM**, grade 12 students from **STEM ACQUAVIVA** of Ateneo de Zamboanga University SHS. We are conducting a quantitative research study titled **“Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students.”** We are writing to invite you to participate as a valued respondent in our study.

Our study aims to assess the health superstition level and health-seeking behaviors among the students. Specifically, we seek to explore how health superstitions influence health-seeking behaviors. As a student, you may have encountered or observed certain beliefs or practices related to health that might be based on superstitions but are not scientifically proven. By gathering information from diverse participants, we aim to gain profound knowledge and understanding of these dynamics and their impact on students' health care practices. Your valuable contribution will provide essential data and enhance our understanding of how health superstitions affect health-seeking behaviors among students. We assure you that your participation is vital in enriching our research.

To ensure your comfort and convenience, we have designed the study procedure to involve survey questionnaires which will be distributed in your respective classroom. The survey questionnaires will be collected the day after it is distributed in order for you to answer without time pressure. Participation in this data gathering is completely voluntary, and you are allowed to withdraw at any time without facing consequences. Your decision will be respected and treated with utmost confidentiality.

Although the study is designed to reduce any possible risks or discomforts, it is important to note that some questions may bring discomfort or cause you to be hesitant when answering the questionnaire, especially those questions related to health-seeking behaviors. Nonetheless, the researchers will ensure privacy and confidentiality when handling your data. This will be achieved by securing the papers in a folder, implementing restricted access so that only the researchers can access the documents, and shredding the survey questionnaires once the data has been analyzed. We want to be transparent about this possibility and assure you that your comfort and privacy are our top priorities. Rest assured that the researchers will return to your respective classroom after a day to collect the survey questionnaires, thus, you will no longer need to hand in the survey questionnaire yourself.

If you have any question, concern or any clarification, do not hesitate to contact us at sh230656@adzu.edu.ph. It will be our pleasure to provide you with further information.

Thank you for the opportunity to contribute to this important research. We are looking forward to your response.

Kindest regards,

AISHA RAFIAH I. ABDUA
KIMBERLY H. HAMISANI

YUZRA H. JUMLAJIL
ABEGAIL GEORGIA H. MAHILUM

Noted by:

NOVA JOY J. SALAJUG, LPT
MAT 121 Research Instructor

SYLVESTER M. SANSON
STM 126 Instructor

CONSENT

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature _____ Date _____

Investigator's signature _____ Date _____

CONSENT FORM

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature: _____ Date: _____

Investigator's signature: _____ Date: _____

CONSENT FORM

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature: _____ Date: _____

Investigator's signature: _____ Date: _____

CONSENT FORM

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature: _____ Date: _____

Investigator's signature: _____ Date: _____

CONSENT FORM

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature: _____ Date: _____

Investigator's signature: _____ Date: _____

CONSENT FORM

I have read and understand the provided information and have the opportunity to ask questions. I understand that my participation is voluntary and that I am free to withdraw at any time, without giving a reason and without cost. I understand that I will be given a copy of this consent form. I voluntarily agree to take part in this study.

Participant's signature: _____ Date: _____

Investigator's signature: _____ Date: _____

ASSENT FORM

I, (Parent/Legal Guardian's name) _____ (optional)

- ☐ hereby give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,
☐ hereby do not give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,

(Student's name) _____ (optional), partake in answering a survey questionnaire regarding the research study titled "Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students." I understand that my child's participation is voluntary and that my child is free to withdraw at any time, without giving a reason and without cost.

Parent/Guardian's Signature: _____ Date: _____

ASSENT FORM

I, (Parent/Legal Guardian's name) _____

- ☐ hereby give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,
☐ hereby do not give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,

(Student's name) _____ partake in answering a survey questionnaire regarding the research study titled "Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students." I understand that my child's participation is voluntary and that my child is free to withdraw at any time, without giving a reason and without cost.

Parent/Guardian's Signature: _____ Date: _____

ASSENT FORM

I, (Parent/Legal Guardian's name) _____

- ☐ hereby give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,
☐ hereby do not give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,

(Student's name) _____ partake in answering a survey questionnaire regarding the research study titled "Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students." I understand that my child's participation is voluntary and that my child is free to withdraw at any time, without giving a reason and without cost.

Parent/Guardian's Signature: _____ Date: _____

ASSENT FORM

I, (Parent/Legal Guardian's name) _____

- ☐ hereby give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,
☐ hereby do not give my consent to the student researchers (Abdua, Hamisani, Jumalil, & Mahilum) to let my child,

(Student's name) _____ partake in answering a survey questionnaire regarding the research study titled "Influence of Health Superstitions on the Health-Seeking Behaviors of Ateneo de Zamboanga University Senior High School Students." I understand that my child's participation is voluntary and that my child is free to withdraw at any time, without giving a reason and without cost.

Parent/Guardian's Signature: _____ Date: _____

APPENDIX C: Validation Letter with Comments and Signature

Ateneo de Zamboanga University
SENIOR HIGH SCHOOL
RESEARCH DEPARTMENT
La Purisma Street, Zamboanga City 7000 (092) 393 0471 local 3000

August 21, 2024
Lady Ivy B. Mañalac
School Nurse

Request for Validation of Survey Questionnaire

Dear Ma'am Ivy,

We are students from STEM – ACQUAVIVA. We are reaching out to ask for your help in validating the survey questionnaire we have developed for our current research. Given your experience as a school nurse, your input would be very valuable to us.

We would appreciate your feedback on the appropriateness, design, and clarity of our questionnaire. Any suggestions you have for improving the data collection method would be highly welcomed.

Please find attached the research title, objectives, and the survey questionnaire we have provided. If you could share your thoughts by **August 22, 2024**, it would greatly help us stay on track.

Thanks for considering this request. Your insights will really help strengthen our research.

Best regards,
Abdua Aisha Rafiah I.
Hamisani, Kimberly H.
Jumlail, Yuzra H.
Mahilum, Abegail Georgia H.
sh230532@adzu.edu.ph

Noted by:
NOVA JOY R. SALAJUG
Research Teacher

Ateneo de Zamboanga University
SENIOR HIGH SCHOOL
RESEARCH DEPARTMENT
La Purisma Street, Zamboanga City 7000 (092) 393 0471 local 3000

VALIDATION CERTIFICATE

This certifies that the survey questionnaire questions for the research titled:

Influence of Health Superstitions on the Health-seeking Behaviors of Ateneo de Zamboanga University Senior High School Students

Conducted by:

Abdua, Aisha Rafiah I.
Hamisani, Kimberly H.
Jumlail, Yuzra H.
Mahilum, Abegail Georgia H.
12 Acquaviva

Have been reviewed and validated by:

Lady Ivy B. Mañalac
School Nurse

Date of Validation: August 21, 2024

Validator's Signature:

Ateneo de Zamboanga University
SENIOR HIGH SCHOOL
RESEARCH DEPARTMENT
La Purisma Street, Zamboanga City 7000 (092) 393 0471 local 3000

SURVEY QUESTIONNAIRE

Survey Title: **Influence of Health Superstitions on the Health-seeking Behaviors of Ateneo de Zamboanga Senior High School Students**

Research Question	Question Type	Survey Question
Personal Details and Other Data	Demographic Questions	- Name (optional) - Grade Level - Section
1. What is the health superstition level of the students? Variables: - Belief in Health Superstitions - Influence of Superstitions on Health Decisions - Common Health Superstitions	☐ Likert Scale ☒ Multiple-Choice ☐ Dichotomous ☐ Rating Scale	1. Do you know any health-related superstitions? a. Yes b. No 2. How did you know about health-related superstitions? a. Family traditions b. Friends c. Community d. Media (books, TV, internet) 3. Do you believe in health-related superstitions? a. Not at all b. Slightly c. Moderately d. Strongly e. Very Strongly <i>change choice not clear</i> 4. How often do you consider health superstitions in making health-related decisions? a. Never b. Rarely c. Sometimes d. Often e. Always 5. Do you believe that health-related superstitions can have a beneficial impact on your well-being? a. Yes, definitely. b. Yes, somewhat. c. No, not really. d. No, not at all.

Ateneo de Zamboanga University
SENIOR HIGH SCHOOL
RESEARCH DEPARTMENT
La Purisma Street, Zamboanga City 7000 (092) 393 0471 local 3000

2. What are the health-seeking behaviors of the students? Variables: - Response to illness - Frequency of Doctor Visits - Preference for Medical Advice - Use of Health Services - Preventive Health Measures	☐ Likert Scale ☒ Multiple-Choice ☐ Dichotomous ☐ Rating Scale	1. How often do you visit a doctor for regular check-ups? a. Once a year b. Every few months c. Every 6 months d. Only when I feel unwell e. Never 2. (Where) do you usually seek health advice when you feel unwell? a. Healthcare professionals (doctors, nurses, etc.) b. Family and friends c. Online resources (websites, apps, forums) d. Traditional or alternative practitioners 3. What is your FIRST course of action when you experience mild symptoms like a headache or a cold? a. Visit the infirmary or health center facilities like infirmary or health center b. Wait for the symptom to improve by itself - <i>check it out</i> c. Seek home remedies or traditional medicine 4. Do you engage in preventive health measures (e.g., vaccinations, health screenings)? a. Never b. Rarely c. Sometimes d. Often e. Always 5. Do you perform actions that are NOT KNOWN to be scientifically proven (e.g., taking a shower after getting soaked with rain to prevent a cold or fever)? a. Never b. Rarely c. Sometimes d. Often e. Always
---	---	--

Ateneo de Zamboanga University
 SENIOR HIGH SCHOOL
 RESEARCH DEPARTMENT
1010 Main Street, Zamboanga City 6000 Philippines (052) 334-1010

3. How do health superstitions influence the health-seeking behaviors of the students?

☐ Likert Scale
☒ Multiple-Choice
☐ Dichotomous
☐ Rating Scale

Variables:

- Frequency of considering health superstitions
- Response to health superstitions
- Situations in which health superstitions are followed
- Perceived influence of health superstitions

1. Do health-related superstitions prevent you from seeking medical help?

- Yes, definitely.
- Yes, somewhat.
- No, not really.
- No, not at all.

2. How often do health superstitions prevent you from seeking medical help?

- Never
- Rarely
- Sometimes
- Often
- Always

3. How do health superstitions influence your choice of treatment for an illness?

- They have no influence in my treatment choices.
- They occasionally influence my treatment choices (i.e., they sometimes lead me to choose home remedies over medical treatments).
- They strongly determine my treatment choices (i.e., they lead me to avoid treatments recommended by doctors).

4. Do health superstitions make you more likely to choose alternative or traditional medicine over conventional medical treatment?

- No, I always prefer conventional medicine.
- I sometimes consider alternative or traditional medicine.
- I often prefer alternative or traditional medicine.
- I exclusively use alternative or traditional medicine.

Note: After consulting with Ms. Ivy, the researchers proceeded with Ms. Salajug as their research validator. The validation feedback was provided orally rather than in written form. However, the researchers carefully documented the key points discussed, which included enhancing the research instrument, adopting standardized research questions as a basis, and eliminating Research Question No. 3 from the questionnaire, as its response could be determined through the interpretation of the correlation statistical results of Questions 1 and 2.

REFERENCES

- Agraan, S. V., Echavez, L. M., Salido, K. J. R., Tapia, M. S., & Sambajon, C. A. (2024). Level of awareness, extent of fulfillment of needs, and impact on health practices concerning adolescent health and development program among senior high school students from De La Salle Medical and Health Sciences Institute. GreenPrints. https://greenprints.dlshsi.edu.ph/grade_12/613/
- Aranha, P. R., Lobo, M. R., & Patil, S. (2022). *Assessment of Health-Seeking Behavior among Undergraduate Students at a University. Journal of Health and Allied Sciences NU, 12(04), 385–391.* <https://doi.org/10.1055/s-0041-1741413>
- Ateneo de Zamboanga University – Ateneo de Zamboanga University Website. (n.d.). <https://adzu.edu.ph/>
- Belief. (2024). In *Merriam-Webster Dictionary*. <https://www.merriam-webster.com/dictionary/belief>
- Bhandari, M. S., & Chataut, J. (2020b). Health Seeking Behaviour among Medical Students Teaching Hospital of Nepal: A Descriptive Cross-sectional Study. *Journal of Nepal Medical Association, 58(221).* <https://doi.org/10.31729/jnma.4741>
- Causal Research (Explanatory research). (n.d.). Research-Methodology. <https://research-methodology.net/causal-research/>
- Cuthino, F. L., Sheilini, M., & Chandrababu, R. (2021). The comparison of health literacy between health science and non-health science members-A cross sectional study of multifaceted public health affair. *Clinical Epidemiology and Global Health, 12,* 100893. <https://doi.org/10.1016/j.cegh.2021.100893>
- Education, social status, and health.* (n.d.). Google Books.

https://books.google.com.ph/books?hl=en&lr=&id=Hqw0DwAAQBAJ&oi=fnd&pg=PT8&dq=importance+of+health&ots=0nCCefkHr9&sig=4ZSFRI8kb2RMYco9CrV_WdXbVJ8&redir_esc=y#v=onepage&q=importance%20of%20health&f=false

Emmanuel, Pratik & Isiaq, Olufemi. (2024). Studying the Impact Of D.P.SL Model on Online Identity Management. European Conference on Social Media. 11. 59-67. 10.34190/ecsm.11.1.2377.

Gunarathna, M. (2023). Health Information Seeking Behaviour in University Students Sri Lanka. *International Journal For Multidisciplinary Research*. <https://doi.org/10.36948/ijfmr.2023.v05i06.11295>

Huamán Fernández, J. R., Rival, G. M. J. G., Cabasisi, K. K. E., & Tabaosares, M. C. (2024). Student Healing Choices: A survey of Senior High School Grade 12 Preferences. *International Journal of Science and Management Studies*, 123–130. <https://doi.org/10.51386>

Imran, H., Sarver, A., Khan, M. U., & Khanum, S. (2024). Exploring Superstitious Beliefs Among Educated Urban Population. *Academy of Education and Social Sciences Review*. <https://doi.org/10.48112/aessr.v4i1.664>

Hussain, N., Clark, A., & Innes, A. (2024). Cultural myths, superstitions, and stigma surrounding dementia in a UK Bangladeshi community. *Health & Social Care in the Community*, 2024, 1–11. <https://doi.org/10.1155/2024/8823063>

Lao, G. (2023). Perceptions of Millennials on Filipino superstitious belief. *www.academia.edu*. https://www.academia.edu/en/73361067/Perceptions_of_Millennials_on_Filipino_Superstitious_Belief

- Lu, J., Luo, M., Yee, A., Sheldenkar, A., Lau, J., & Lwin, M. O. (2019). Do superstitious beliefs affect influenza vaccine uptake through shaping health beliefs? *Vaccine*, 37(8), 1046–1052. <https://doi.org/10.1016/j.vaccine.2019.01.017>
- Maritim, P., Silumbwe, A., Zulu, J. M., Sichone, G., & Michelo, C. (2021). Health beliefs and health seeking behavior towards lymphatic filariasis morbidity management and disability prevention services in Luangwa District, Zambia: Community and provider perspectives. *PLoS Neglected Tropical Diseases*, 15(2), e0009075. <https://doi.org/10.1371/journal.pntd.0009075>
- Milton, R., Alkali, F. I., Modibbo, F., Sanders, J., Mukaddas, A. S., Kassim, A. B. H., Sa'ad, F. H., Tukur, F. M., Pell, B., Hood, K., Ghazal, P., & Iregbu, K. (2021). A qualitative focus group study concerning perceptions and experiences of Nigerian mothers on stillbirths. *BMC Pregnancy and Childbirth (Online)*, 21(1). <https://doi.org/10.1186/s12884-021-04207-4>
- Oberoi, S., Chaudhary, N., Patnaik, S., & Singh, A. (2016). *Understanding health-seeking behavior*. Journal of family medicine and primary care. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC5084583/>
- Pomona Valley Health Centers. (2023, November 11). The importance of regular Check-Ups. <https://mypvhc.com/importance-regular-check-ups/>
- Sazonets, I., Yekimov, S., Hinke, J., Šálková, D., & Křečková, R. (2024). Spearman's correlation coefficient: The case of duplicate ranks. *E3S Web of Conferences*, 538, 02031. <https://doi.org/10.1051/e3sconf/202453802031>

Sheikh, M. A., Cherian, B., Gavande, S. V., Kutlehria, S. K., & Hussain, T. (2022). PRE-EXPERIMENTAL STUDY TO ASSESS THE EFFECTIVENESS OF STRUCTURED TEACHING PROGRAMME ON KNOWLEDGE REGARDING HOME REMEDIES USED IN DYSMENORRHEA AMONG ADOLESCENT GIRLS IN SELECTED HIGHER SECONDARY SCHOOLS DISTRICT KISHTWAR. *International Journal of Advanced Research*, 10(02), 263–271. <https://doi.org/10.21474/ijar01/14203>

Simply Psychology. (2023, July 31). *Cluster Sampling: Definition, method and Examples*. <https://www.simplypsychology.org/cluster-sampling.html>

Šulinskaitė, K., Zagurskienė, D., & Blaževičienė, A. (2022). Patients' health literacy and health behaviour assessment in primary health care: evidence from a cross-sectional survey. *BMC Primary Care*, 23(1). <https://doi.org/10.1186/s12875-022-01809-5>

Taher, M., Pashaeypoor, S., Cheraghi, M. A., Karimy, M., & Hoseini, A. S. S. (2020). Superstition in health beliefs: Concept exploration and development. *Journal of Family Medicine and Primary Care*, 9(3), 1325. https://doi.org/10.4103/jfmpc.jfmpc_871_19

Team, I. (2023, December 25). *What's the Difference Between Systematic Sampling and Cluster Sampling?* Investopedia. <https://www.investopedia.com/ask/answers/051815/what-difference-between-systematic-sampling-and-cluster-sampling.asp#:~:text=Cluster%20sampling%20is%20relatively%20cheap,sample%20from%20within%20that%20cluster.>

Thomas, L. (2023, June 22). *Cluster Sampling | A Simple Step-by-Step Guide with Examples*. Scribbr.

<https://www.scribbr.com/methodology/cluster-sampling/#:~:text=What%20is%20cluster%20sa>

mpling%3F,the%20population%20as%20a%20whole.

Tung, H., Chen, Y., Chen, C., Gee, M., Muo, C., & Chiu, S. (2023). How cultural behaviors and superstitions associate the willingness to undergo cataract surgery in Taiwan: a nationwide survey. *Medicina-lithuania*, 59(5), 973. <https://doi.org/10.3390/medicina59050973>

Uwayezu, D., Ntigura, E., Gatarayiha, A., Erem, A. S., Haque, M., Majumder, M. A., & Razzaque, M. S. (2022). Conflict between Science and Superstition in Medical Practices. *International Medical Education*, 1(2), 33-42. <https://doi.org/10.3390/ime1020007>

Widayanti, A. W., Green, J., Heydon, S., & Norris, P. (2020). Health-Seeking behavior of People in Indonesia: A narrative review. *Journal of Epidemiology and Global Health*, 10(1), 6. <https://doi.org/10.2991/jegh.k.200102.001>

World Health Organization. (n.d.). *Constitution*. Retrieved February 8, 2024, from <https://www.who.int/about/accountability/governance/constitution>

World Health Organization. (2023, November 17). *Traditional medicine*. Retrieved February 8, 2024, from <https://www.who.int/news-room/questions-and-answers/item/traditional-medicine>

World Health Organization: WHO. (2019, November 25). *Traditional, complementary and integrative medicine*. <https://www.who.int/health-topics/traditional-complementary-and-integrative-medicine#tab>

Yimaz, F. A., & Uzunçakmak, T. (2023). Pregnant Women's Attitudes towards Complementary and Alternative Medicine and the Effect of their Superstitions on their

Psychosocial Health

Levels.<https://www.semanticscholar.org/paper/Pregnant-Women%E2%80%99s-Attitude-towards-Complementary-of-Yimaz-Uzun%C3%A7akmak/dc38a910a7f0c965deda9abd690c07aa538cdf9>

Zhang, Q. (2015). *Traditional and complementary medicine in primary health care*. Health for All - NCBI Bookshelf. <https://www.ncbi.nlm.nih.gov/books/NBK316267/>